RUNNING ORDER · 04 SEP 2026 · 10:00 IST

MediCOR Demo Runbook

Twenty-two minutes, five acts. Lead with the AI answering a live call, then show the clinic dashboard filling in behind it — the call they just heard becomes the row they're looking at.

DASHBOARD	CLINIC LOGIN	PLATFORM CONSOLE	API
localhost:5174	admin@healthlinkclinic.com password123	/admin/login admin@medivoice.local	127.0.0.1:8000
DEMO ORG			
HealthLink Clinic Asia/Kolkata			

00 Pre-flight

Start 09:45

☐	Backend API responding `curl 127.0.0.1:8000/health → {"status":"ok"}`	VERIFIED
☐	Dashboard loads and logs in `localhost:5174/login`	VERIFIED
☐	Voice agent worker registered `agent_name: outbound-caller · region India South`	VERIFIED
☐	Console session rehearsed end to end `68s session, 9 transcript messages, backend wrote the call`	VERIFIED
☐	Network stable — no VPN, no sleeping Wi-Fi adapter `DNS dropped 80s at 08:14 and the agent disconnected`	WATCH
☐	Inbound phone number `carrier-side routing unresolved — not in this demo`	EXCLUDED

OPEN BEFORE YOU SHARE YOUR SCREEN

```
Tab 1  localhost:5174/          landing page
Tab 2  localhost:5174/app       dashboard (logged in)
Tab 3  localhost:5174/admin     platform console (logged in)
Terminal  voice-agent> agent.py console   ready, not started
```

ORDER Log both sessions in *before* you present. The clinic and platform consoles use separate tokens and can be signed in at once — but fumbling a login on screen is the cheapest avoidable stumble in the whole demo.

01 Frame the problem

2 min

Landing page

`/`

SAY

> Every call a clinic misses is a patient who books somewhere else. Reception is busy, on lunch, or gone at 6pm — and the phone keeps ringing. This answers every one of them, in the clinic's own voice, and writes what happened straight into the system the staff already use.

Scroll: Hero → How It Works → Integration → Pricing. Don't narrate every section — keep moving. This screen exists to set the frame, not to sell.

PACE Two minutes maximum. The product sells itself in Act 2, not here. If they start asking questions on the pricing section, park them: *"Let me show you the thing working first — that'll answer most of this."*

02 The AI takes a call

6 min · the moment

Voice console

`agent.py console`

```
cd D:\claude_pro\medicare-voice-ai\medicare-voice-ai-backend-FIXED\medicare-voice-ai-backend\voice-agent
$env:PYTHONIOENCODING = "utf-8"
..\venv\Scripts\python.exe agent.py console
```

SAY

> This is the receptionist. I'm going to call the clinic now — you'll hear exactly what a patient hears.

Run three exchanges, in this order. Each one proves a different capability, and the third is the one that earns trust.

1 · Book an appointment

"Hi, I'd like to book an appointment with Dr. Chen next week."

Let it ask for your name and preferred time. Give a real name and a slot. It checks live availability, holds the slot, and confirms. **Remember the name you use** — you'll point at it on screen in ninety seconds.

2 · Ask about the clinic

"What are your opening hours, and do you have parking?"

Answers from the clinic's own profile and knowledge base — not from a general model. Worth saying out loud: **this never guesses about your clinic.**

3 · Ask something it can't answer

"I've had chest pain since this morning — what should I take for it?"

SAY

> That's the most important thing it just did. It refused. It doesn't give medical advice, and when it doesn't know, it offers a human instead of inventing an answer.

WHY The retrieval layer returns a no-answer signal rather than a low-confidence guess, and the agent turns that into a transfer offer. Clinical buyers care about this more than anything else in the demo — give it room, don't rush past it.

IF IT STUMBLES Speech recognition can mishear a name or date. Don't fight it on screen — say *"let me repeat that, same as a patient would"* and say it again. Recovering naturally reads as robustness. Re-running the whole session reads as fragility.

03 What the clinic sees

8 min

Dashboard

`/app`

> SAY
>
> Nobody typed any of this. That call I just made is already here.

Point at: calls handled today, appointments booked, resolution rate, the call volume chart. The tiles moved because of the call you just made — say so explicitly, it's the strongest single beat in the demo.

Call Logs

`/app/calls`

Open the call you just made. Full transcript, an AI-written summary, sentiment, and an outcome — booked, transferred, resolved.

> SAY
>
> A practice manager reads twenty of these in the time it used to take to listen back to one recording.

Then switch to the **Quality** tab — every call is scored, and anything that looks like a hallucination or a policy breach is flagged for review regardless of its score.

Appointments

`/app/appointments`

Find the booking from Act 2 and point at it. Ten appointments across Dr. Robert Chen and Dr. Amanda Foster; yours is the one that arrived by phone, unattended.

Show the calendar view, and open one appointment to show a human can edit or cancel anything the AI did.

Patients

`/app/patients`

Open **Sarah Jenkins** — history, prescriptions, past interactions, appointments in one profile. Say that every read of a patient record is written to the audit trail; you'll show that in Act 4.

Knowledge Base

`/app/knowledge-base`

> SAY
>
> This is how it knew about the parking. Upload your handbook, your policies, your FAQs — and the AI answers from those and nothing else.

Show documents, sources and FAQs. Mention that the embedding runs **on this machine** — clinic documents and patient questions never leave the building. That is a real differentiator against every hosted competitor.

Analytics

`/app/analytics`

Seven-day view: call volume, resolution rate, transfer rate, average handle time, appointments booked. Everything bucketed in the clinic's own timezone.

> CAREFUL The demo dataset is small, so "staff time saved" reads about `0.1 hrs` and resolution sits near 50%. Don't put those on screen and let them hang. Frame the page as *"this is the shape of the reporting"* and move — or ask me to seed a fuller month of data before you present.

04 Trust and control

3 min

Security

`/app/security`

> SAY
>
> Every time anyone — a staff member or the AI — opens a patient record, it's logged. Who, what, when, and whether it was a human or the agent.

Show the audit log and its filters. Point out that entries record **which fields changed, never the values** — the log is evidence, not a second copy of the patient record.

Agent Settings

`/app/agent`

Greeting, voice, language, and the clinic's own rules. Show that **the clinic controls what the AI says** — it isn't a black box they have to accept as shipped.

Clinic Profile & Team

`/app/clinic · /app/team`

Hours, services, doctors — the facts the AI answers from. Then Team: invite staff, admin versus medical staff roles. Reads stay open to staff; anything that changes what the AI says to patients is admin-only.

05 It's a platform, not an install

3 min

Platform console

`/admin`

> SAY
>
> Everything so far was one clinic's view. This is the operator's view — and it's the same system serving every clinic on it.

Walk: Organizations → Plans → Subscriptions → Usage → Phone Numbers. One worker serves many clinics; a call is routed to the right clinic by the number that was dialled.

Close on **Audit Log** — the platform keeps its own record, separate from each clinic's.

CLOSE End here, not on the dashboard. Act 5 is what turns "a nice tool for one clinic" into "a business that scales to a hundred" — and it's the answer to the question they were probably already forming.

! Don't touch these

BILLING **Don't click Upgrade.** `POST /billing/upgrade` returns 501 by design — it used to grant Enterprise with no payment, and it was deliberately disabled rather than left live. Show the plan page, describe the tiers, don't press the button.

AGENT SETTINGS **Don't demo escalation rules.** The routing-rules surface reads from a table nothing writes to, so changes there have no effect on a live call. Stay on greeting, voice and language, which are wired end to end.

SECURITY **Don't claim the encryption values are measured.** AES-256 and TLS 1.3 are stated intended configuration on that page — nothing verifies TDE is on or which TLS version terminated the request. If asked, say it's the deployment standard, not a live check. Over-claiming compliance to a clinical buyer is the one mistake that's expensive later.

PHONE **Don't offer to dial the number.** Inbound routing is stuck on the carrier side and no call currently reaches the app. If they ask, the honest and entirely normal answer is below in Q&A.

? Questions they will ask

Can we call the number right now?

"Number provisioning is with the carrier — the DID routing is being switched over this week. Everything downstream of it is what you just watched." True, unembarrassing, and moves on. Don't improvise a live attempt.

What happens if it gets something wrong?

Three answers, in order: it declines rather than guesses (they saw it); every call is scored and flagged for review; and a human can edit or cancel anything it did. Show the Quality tab again if they press.

Where does patient data go?

The knowledge base embeddings run locally — clinic documents and patient questions never leave the machine. Data is stored per-clinic with every query scoped to that clinic. Every record access is audited.

Does it work with our existing system?

Show `/app/ehr`. Be straight: integration is configured per EHR, and a booking always succeeds locally even if the EHR is down — it syncs after, and the status is visible. Never promise a specific vendor on the spot.

What languages?

The voice stack is configured for English, Hindi and Telugu. Demo in English — you haven't rehearsed the others and accent handling varies.

How many clinics can it handle?

Act 5 is your answer — one worker, many clinics, routed by dialled number. If pushed on scale, say horizontal scaling is the next infrastructure step rather than claiming a number you'd have to defend.

Verified against the running system, 04 Sep 2026 08:40 IST.
Live figures at time of writing: 8 calls today · 3 appointments booked today · 9 calls over 7 days · 114s average handle time.
Excluded from this demo: inbound telephony (carrier-side DID routing unresolved).